

PRIOR AUTHORIZATION APPEAL

Formal Medical Appeal — Oncology

TO:	UnitedHealthcare — UnitedHealthcare Medical Director — Prior Authorization Appeals
DATE:	March 26, 2026
RE: Drug:	Tagrisso (osimertinib)
Denial Type:	STEP THERAPY
Reference #:	PA-2026-5582910
Patient ID:	PT-2026-002
Member ID:	UHC-OSI-XXXX

ATTACHMENTS ENCLOSED

1. NCCN NSCLC v4.2026 — NSCLC-18 p.9
2. FDA Package Insert — Tagrisso (NDA 208065)
3. Prior Therapy Summary
4. Original Denial Notice dated 2026-03-11
5. Patient Biomarker Report

I. EXECUTIVE SUMMARY

This appeal requests immediate reversal of UnitedHealthcare's denial of Tagrisso (osimertinib) for a patient with Stage IV lung cancer. The denial, citing "step therapy", directly contradicts NSCLC v4.2026 Category 1 recommendation and FDA approval for this indication.

The requested therapy is FDA-approved specifically for this patient population. The conflict has been identified as Type C: UHC step therapy requires erlotinib/gefitinib before osimertinib. NCCN Category 1 designates osimertinib as preferred FIRST-LINE. FLAURA trial showed superior OS with osimertinib vs first-gen TKIs. Requiring inferior prior therapy contradicts NCCN sequence..

This denial constitutes bad-faith delay of life-saving oncology treatment and may violate New York Insurance Law § 2601 (unfair settlement practices) and NY Public Health Law § 4903 (utilization review requiring medically necessary care approval within 72 hours for urgent requests).

II. CLINICAL BACKGROUND

The patient presents with Stage IV lung cancer with the following biomarker profile: {"EGFR":"L858R","PDL1":"40%","ALK":"negative"}. Prior therapies include: []. Current performance status: ECOG 1.

Given the documented disease progression and biomarker profile, Tagrisso (osimertinib) represents the evidence-based, guideline-concordant next line of therapy. No alternative therapies in the same class are available that match this patient's molecular profile.

Delaying access to this therapy risks disease progression, metastatic spread, and irreversible harm to this patient's survival outcomes.

III. NCCN & FDA GROUND TRUTH — CLINICAL STANDARDS

The NSCLC v4.2026 — the authoritative oncology treatment standard — designates osimertinib as a Category 1 recommendation (preferred) for this exact indication: EGFR-mutant NSCLC first-line in patients with biomarker profile {"EGFR":"ex19del or L858R"}.

NCCN Category 1: Osimertinib is the preferred first-line therapy for EGFR exon 19 deletion or L858R substitution NSCLC.

FDA approval was granted for FDA approved for first-line EGFR ex19del or L858R NSCLC (Apr 2018) and T790M-positive (Nov 2015). (Reference: NDA 208065). This approval constitutes the highest regulatory standard of evidence for efficacy and safety in this population.

NCC Category 1 indicates uniform consensus based on high-level evidence — there is no credible clinical or scientific basis for the payer's denial classification.

%¶ **NCCN CITATION**

NCCN NSCLC v4.2026 — NSCLC-18 p.9

IV. PAYER POLICY CONTRADICTION ANALYSIS

UnitedHealthcare policy MP-ONC-4102 states: "First-line coverage requires trial of first-generation EGFR TKI (erlotinib/gefitinib) unless T790M+ at diagnosis.". This language directly contradicts:

1. FDA-approved indication for Tagrisso covering this exact patient population.
2. NSCLC v4.2026 Category 1 recommendation.
3. The peer-reviewed clinical evidence base underlying both of the above.

Conflict type C: UHC step therapy requires erlotinib/gefitinib before osimertinib. NCCN Category 1 designates osimertinib as preferred FIRST-LINE. FLAURA trial showed superior OS with osimertinib vs first-gen TKIs. Requiring inferior prior therapy contradicts NCCN sequence.

This denial appears to be a systematic policy that creates coverage barriers for guideline-concordant oncology care — a pattern that has been identified across multiple plans and geographies. Such systematic barriers may constitute bad-faith insurance practices.

V. LEGAL FRAMEWORK & BAD-FAITH ANALYSIS

Under ERISA Section 502(a)(1)(B) (29 U.S.C. § 1132), the patient has the right to recover benefits wrongfully denied under the plan. The plan's denial, which contradicts NCCN Category 1 guidelines and FDA regulatory standards, exceeds the plan's lawful authority to define medical necessity contrary to established clinical evidence.

Applicable state law: New York Insurance Law § 2601 (unfair settlement practices) and NY Public Health Law § 4903 (utilization review requiring medically necessary care approval within 72 hours for urgent requests)

"This denial contradicts NSCLC v4.2026, NSCLC-18 p.9. Continued denial constitutes bad-faith delay of life-saving oncology treatment and may constitute a violation of New York Insurance Law § 2601 (unfair settlement practices) and NY Public Health Law § 4903 (utilization review requiring medically necessary care approval within 72 hours for urgent requests)."

NOTICE TO MEDICAL DIRECTOR:

Continued denial of this FDA-approved, NCCN Category 1 therapy constitutes bad-faith delay of life-saving oncology treatment. This notice serves as formal documentation of bad-faith practices under applicable state and federal law.

VI. REQUESTED RESOLUTION

We respectfully demand immediate approval of Tagrisso (osimertinib) for the above-referenced patient and reversal of denial reference PA-2026-5582910. This request should be treated as urgent given the oncologic context.

Per ERISA regulations and applicable state law, a decision must be rendered within 72 hours for urgent/ expedited appeals or 30 days for standard appeals. Failure to respond within this timeframe will be treated as a deemed denial and will be escalated to the appropriate state insurance regulatory authority and, if necessary, federal courts.

Respectfully submitted,

Authorized Clinical Representative

Generated by PAE-Onc Appeal Engine | 2026-03-26T04:53:16.475Z | Appeal ID: AP-2026-D2FJIMKI

